

BACKGROUND WORLD HEALTH ORGANIZATION

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Introduction

"Nutrition negligence during childhood and its Impacts on Adulthood."

Greetings, delegates, and be welcome to the World Health Organization. We, as a Chair, are truly honored and grateful to have you as members of this committee; we firmly believe in your capacity and know you will do an outstanding job.

This Model of the United Nations is a great development opportunity to take part in for you, as a delegate, as it will allow you to develop skills such as debating any sort of topic, information seeking, gathering and filtering, improving public speech and time organization.

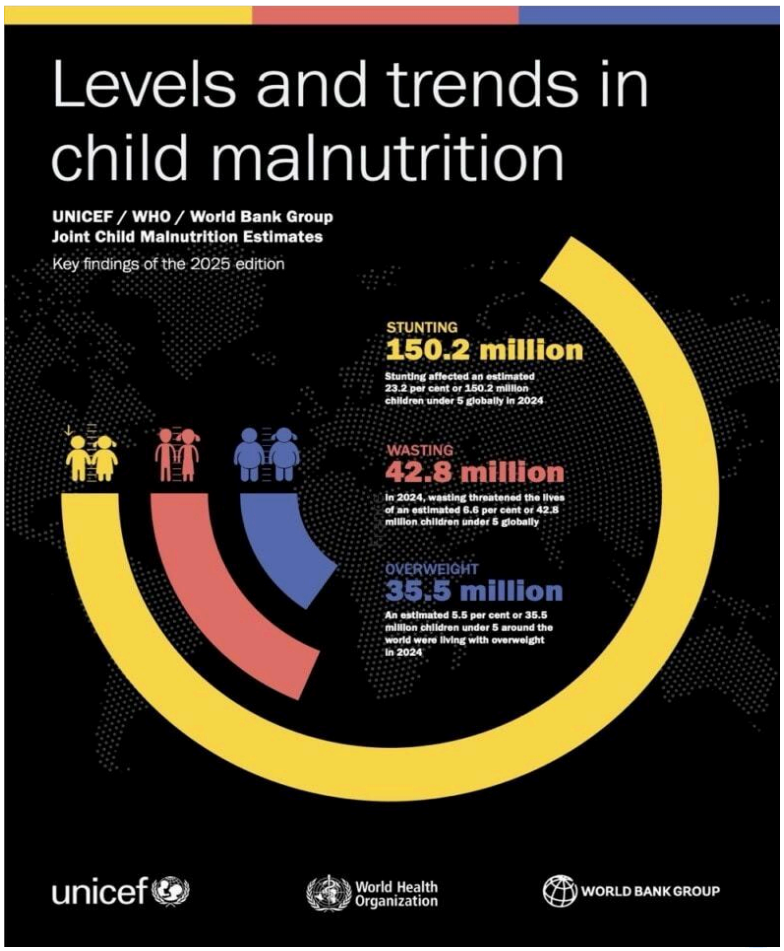
Our job as a Chair is to make sure delegates are prepared to give all their effort in order to make this a memorable experience in their life; we will guide them through the entire model with the purpose of preparing them for a debate where delegates will be able to form ideas, offer solutions to the problem at hand and establish a dialogue with others to reach an agreement within everyone's opinions.

In the World Health Organization, delegates' participation in this committee will be invaluable. It will be their job to use their point of view, empathy, and analytical skills to reach a solution in which global health is the priority. This document contains this committee's history, long and short terms of the topic, the position of the United Nations, controversy points, our objective, and the conclusion; everything needed to make the most out of this model. Your voice is your identity, and it is time you use it in order to improve the world.

The topic "Nutrition Negligence and its Impact on Adulthood" has been chosen since it is a problem that has affected global health for years and is still a major concern in modern society. Nutrition negligence is a problem that has been present in human society since its formation; this problem occurs when an individual does not acquire enough nutritional value through the food they consume, resulting in people not having sufficient knowledge on how to stay healthy.
This problem can be visualized with the growing amounts of children and adults who are stunted (too short for age), wasted (too thin for height), or overweight.

The Universal Declaration of Human Rights, Art. 25, establishes that *everyone has the right to a standard of living adequate for the health and well-being of themselves and of their family, including food, clothing, housing, medical care, and necessary social services.*

Around 45% of deceased children under 5 years of age are linked to undernutrition, mostly occurring in developing countries. There are many elements in a child's environment that allow for malnutrition to exist and prevail all throughout their life; elements that include developmental, economic, social and medical aspects that severely damage the health of the individual and affect their enjoyment of life as a whole.



Approximately 7 people pass away from malnutrition every minute. The burden imposed by these losses extends beyond the sad story of young lives being lost. They nullify the capacity of children to grow and experience life, and of young adults to work and raise a family. On the other hand, these losses exhaust resources that would, in other cases, be utilized for improving the human condition.

Furthermore the consequences of neglecting nutrition in the earliest stages of life are severe and last a lifetime, studies show that adults who suffered malnutrition as a child, have shown higher propensity to non-communicable diseases later in life, including diabetes and heart disease, this is worsened further by weak eating habits, social and cultural ignorance, as well as economical imbalance in prices of nutritious food opposed to the cost of fast food.

This problematic is present in many countries around the world, each with its unique details and solutions. However, the lack of information regarding eating habits in children, parents not having enough resources to feed their children, and many other factors make developing countries in South America, Africa and Asia, one of the main places where support is needed.

On the other hand, developed countries also face a difficult situation as obesity in both children and adults is a common constant in nations that have a strong economy but a weak culture regarding healthy eating habits.

History of the Committee

Foundation

Founded in 1948, the World Health Organization (WHO) is the United Nations (UN) agency that connects nations, partners, and people to promote health, keep the world safe and serve the vulnerable so everyone, everywhere can attain the highest level of health.

Immediately after World War II, in 1945, the United Nations Conference on International Organizations located in San Francisco, voted to establish a new international health organization. In April 1945, representatives of Brazil and China proposed that an international health organization should be established and a conference to frame its constitution convened. In 1946, the Economic and Social Council of the UN instructed the Secretary General to summon said conference. A Technical Preparatory Committee met in Paris from March 18th to April 5th, 1946, and drew up proposals for the Constitution, which were presented to the International Health Conference in New York City between June and July of 1946. Derived from these proposals, the Conference drafted and adopted the Constitution of the World Health Organization, signed on the 22nd July 1946 by representatives of 51 members of the UN and of 10 other nations. This Constitution came into force on April 7th, 1948, a date we now celebrate every year as World Health Day.

How the WHO operates

WHO leads global efforts to expand universal health coverage. This committee directs and coordinates the world's response to health emergencies and promotes healthier lives – from pregnancy care throughout old age. WHO outlines an ambitious plan for the world to achieve real health for all, using science-based policies and programs.

With administrative headquarters in Geneva, the governance of WHO operates through the World Health Assembly, which meets annually as the general policy-making body, and through an Executive Board of health specialists elected for three-year terms by the assembly.

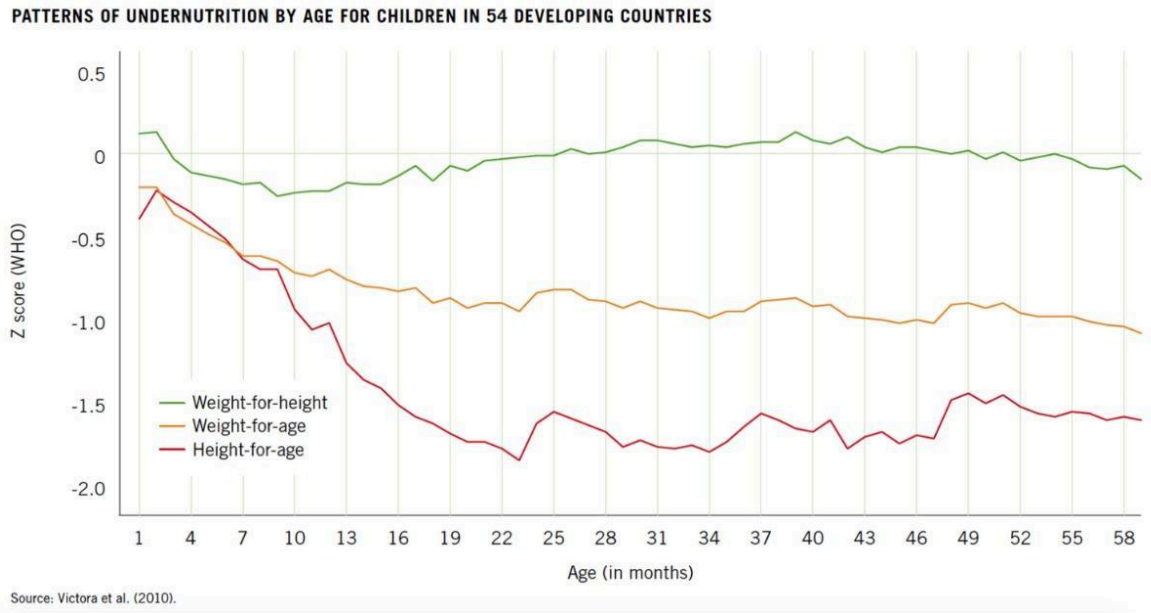
This organization defines health positively as “a state of complete physical, mental and social well-being”, and not merely “the absence of disease or infirmity”, and its priorities include: assisting countries that seek progress toward universal health coverage; helping countries establish their capacity to adhere to International Health Regulations; increasing access to essential and high-quality medical products; addressing the role of social, economic and environmental factors in public health; coordinating responses to non-communicable disease and promoting public health.

WHO actively sponsors measures for the control of epidemic and endemic diseases by promoting mass campaigns involving nationwide vaccination programs, instruction in the use of antibiotics and insecticides, the improvement of laboratory and clinical facilities for early diagnosis and prevention, assistance in providing pure-water supplies and sanitation systems, as well as health education for people living in rural communities. These campaigns have had some success against Acquired Immunodeficiency Syndrome (AIDS), tuberculosis, malaria, and a variety of other diseases.



Long term

Neglect of children's and personal nutrition has been a common threat that has overcome the barrier of time, this is proven with statistics provided by the American Bar Association, which demonstrate that at the end of the 20th century and beginning of the 21st century, obesity in children and other forms of malnutrition increased in 20%, studies showed that the increase in this percentage was due to parents with children in foster care did not give them the necessary care in nutrition at an early age.



In 2007, nutrition was an overlooked aspect when it came to child care; as well as breastfeeding becoming less common due to the rise in popularity of baby formulas, leading to decay in infant health and many health problems early in life, such as atopy, diabetes mellitus, and childhood obesity.

In both developed and developing countries, about 12% of children have a form of disability, one of the most prevalent being malnutrition. On average, only 2-3% of that 12% of children are considered disabled by their community, therefore not receiving proper care and treatment regarding their disability. Nutrition can do much to prevent disabilities and ensure that a disabled child is not at a disadvantage. Urban children from the age of 6 upwards will be vulnerable to shocks from a lack of supervision. Care has to be provided in some form through other communal institutions.

Social consequences

FAO studies from 2014 show that diets that do not provide enough micronutrients have led to serious diseases, including anemia, intellectual retardation, and permanent blindness. The global prevalence of overweight and obesity has increased in all regions, rising from 24% to 34% between 1980 and 2008. Said diseases increase the risk of noncommunicable diseases, such as cardiovascular disease, diabetes, some cancers, and osteoarthritis, posing a significant threat to public health.

An OAS study from 2005 demonstrates that the effects of malnutrition in early childhood (0 to 8 years) can be devastating and long-lasting; they impair behavioral and cognitive development, academic performance, and reproductive health.

Economic consequences

A study conducted in 2007 by the Economic Commission for Latin America and the Caribbean (ECLAC) and the World Food Program (WFP) revealed that, in developing countries, GDP losses can reach between 2% and 3% annually due to malnutrition, even reaching 6%.

A nation's economy can also be affected by diseases caused by malnutrition, as they affect future employment and income opportunities, thus perpetuating the cycle of underdeveloped economic environments.

The economy of a developing family can be affected by malnutrition in children. After a child suffers from an illness caused by malnutrition, the family must spend a lot of money to seek treatment, which becomes costly in the long run. This leads to damage to the family economy and leaves the family in a vulnerable situation.

Political consequences

The nutrition policy landscape has fundamentally changed since the first Lancet series on maternal and child malnutrition was published in January 2008. Since then, nearly every major development agency has published a policy paper on malnutrition.

Official Development Assistance (ODA) for basic nutrition increased from $259 million in 2008 to $418 million in 2011. Moreover, G8 countries reported increases of nearly 50% in bilateral spending on nutrition-specific and sensitive interventions between 2009 and 2011.

This shift is attributable to several factors: the food price spikes of 2007–2008 sparked renewed media and policy interest in malnutrition; the 2008 Lancet series provided policymakers with a set of tangible interventions that were effective in several locations; and the 2008 Copenhagen Consensus concluded that nutritional interventions were among the most cost-effective for development.

Government´s Response

On the 19th of November 2014, over 170 countries made concrete commitments and adopted a series of recommendations on policies and investments aimed at ensuring that all people have access to healthier and more sustainable diets.

Ministers and senior officials responsible for health, food or agriculture and other aspects of nutrition adopted the Rome Declaration on Nutrition, and a Framework for Action, which set out recommendations for policies and programs to address nutrition across multiple sectors. This took place in the Second International Conference on Nutrition (ICN2), organized by the Food and Agriculture Organization of the United Nations (FAO) and WHO.

Short Term

In 2020, stunting affected an estimated 22% or 149.2 million children under the age of 5 years, while wasting affected 6.8% or 45.4 million children under the age of 5 years. Around 45% of deaths among children under the age of 5 years are linked to undernutrition. These mostly occur in low- and middle-income countries. At the same time, in these same countries, rates of childhood obesity and obesity are rising. Every country in the world is affected by one or more forms of malnutrition. Combating malnutrition in all its forms is one of the greatest global health challenges.

Although there has been progress, only one-third of countries are on track to reduce child malnutrition. An estimated 39.5 million children will not meet growth targets by 2030, with more than 80% of these cases concentrated in Africa.

The COVID-19 pandemic affected many countries, especially those in the regions of South Asia and sub-Saharan Africa, in which there has been an ongoing burden of child undernutrition.

Social consequences

In 2020, UNICEF Executive Director Henrietta Fore commented that more than seven months after the first recorded cases of COVID-19, a severe impact was recorded, as households reported scarcity of economic resources and food insecurity rates increased.

Essential nutrition services and supply chains were disrupted. Food prices soared. As a result, the quality of children's diets declined and malnutrition rates increased, negatively impacting people's physiological and mental capacities. This, in turn, reduces productivity levels, affects their cognitive development and learning, and increases their vulnerability to financial insecurity in adulthood, both for themselves and for their respective countries.

In summary, the 2020 COVID-19 pandemic left an immense impact on households' economies and directly affected food insecurity rates, as well

as disrupting children's habits and supply chains. All this greatly affecting the nutrition and diets of many children worldwide.

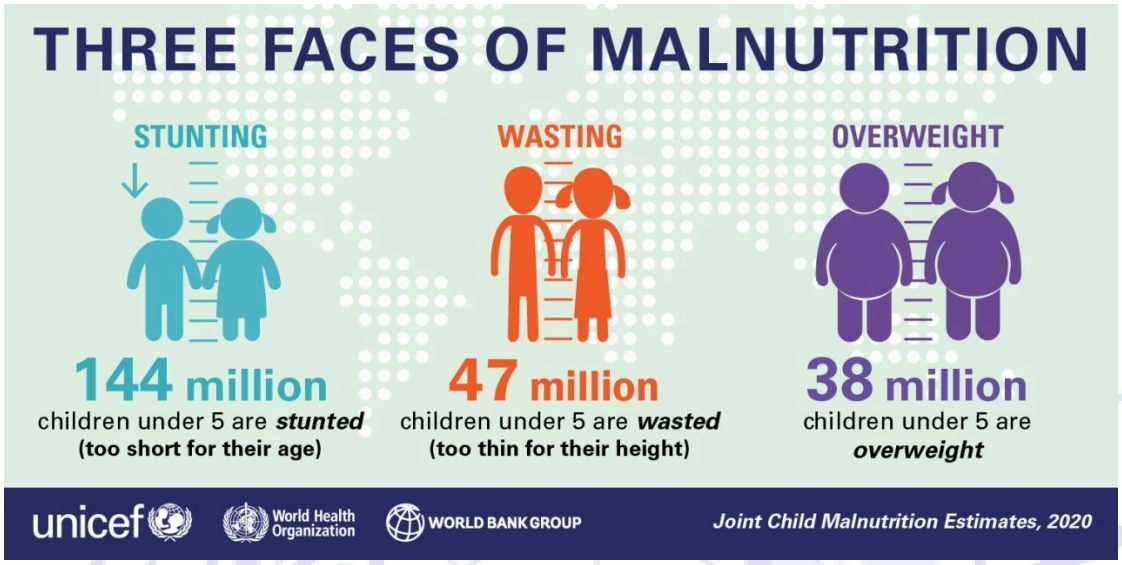
Economic consequences

In 2020, the global market and its economy were greatly affected by the COVID-19 pandemic, which not only caused damage to the world's economy by raising the prices of food, but also affected nutritional health centers for family care, as well as the need to buy medicines to treat the virus arose, leaving families with few resources in a deplorable situation where hunger and lack of financial resources was the constant. This not only affected families but also affected employment and income opportunities, thus perpetuating the cycle of underdeveloped economic environments.

Political consequences

Fighting malnutrition and providing healthcare for all is a main priority; however, without political commitment at the country-level, the policies, programs and resources required to reduce and eliminate malnutrition in all its forms are unlikely to be adopted, effectively implemented and sustained. Recent advances in the political economy of nutrition literature reveal a complex web of factors that drive political commitment across different country contexts and forms of malnutrition.

Most major advancements in creating strong programs and frameworks to tackle nutrition negligence in all its form have a core that is government and organization based, instituting this method needs full political commitment from the country's government, which has not been accomplished in many countries as of 2018, in order to achieve the objectives proposed by the United Nations in the United Nations Decade of Action on Nutrition.



Topic and the United Nations

As an international organization, the World Health Organization is responsible for providing a good quality of life, safety and health care worldwide based on topical analysis and a focus on achieving adequate outcomes. As nutrition is a worldwide topic with many factors and variables, the United Nations and WHO require support from other organizations and collaborators, such as:

UN Organizations and Collaborators

Resolutions regarding this problematic call upon the Food and Agriculture Organization of the United Nations (FAO) and the World Health Organization (WHO) to lead the implementation of said solutions, in collaboration with the World Food Programme (WFP), the International Fund for Agricultural Development (IFAD) and the United Nations Children's Emergency Fund (UNICEF), involving coordination mechanisms such as the United Nations System Standing Committee on Nutrition (UNSCN) and multi-stakeholder platforms such as the Committee on World Food Security (CFS).

It also invites national governments and other stakeholders, including international and regional organizations, civil society, the private sector, and academia, to actively participate.

United Nations Decade of Action on Nutrition

On the 1st of April 2016 in the United Nations Headquarters in New York, the United Nations General Assembly proclaimed a UN Decade of Action on Nutrition that will run from 2016 to 2025 (This campaign has been extended to 2030). FAO welcomed the decision, calling it a major step towards mobilizing action around reducing hunger and improving nutrition around the world.

At the time of this resolution being enacted, nearly 800 million people remain chronically undernourished and over two billion people suffer from micronutrient deficiencies. And the prevalence of people who are overweight or obese is increasing in nearly all countries.

The goals of the campaign include improving the nutritional status of the population in all areas, particularly in early life, by preventing and treating malnutrition among pregnant women and young children.

World Leadership Dialogue on Nutrition

On the 28th of July 2023, WHO, in collaboration with UNICEF, FAO, and WFP, organized the Leadership Dialogue on Food Systems for People's Nutrition and Health at the first United Nations Food Systems Summit Stocktaking Moment.

The dialogue emphasized that the world is off track to meet the global nutrition targets as the pervasive status of all forms of malnutrition persists. Alarmingly, 2.4 billion people suffer from food insecurity, while 670 million adults live with overweight or obesity. Already, the double burden of malnutrition is negatively impacting the health and prosperity of future generations, with 478 million children aged under 5 impacted by stunting, while 145 million 5-9 year olds live with overweight/obesity.

Speakers highlighted the far-reaching implications of our food choices on our health, environment, and society, the importance of a common narrative for healthy diets, and the great need to invest in nutrition and scale up policy action to yield health, planetary, and economic results.

As world leaders gathered to take stock of the progress to transform our food systems and accelerate progress towards the 2030 Sustainable Development Goals (SDG) agenda, the Leadership Dialogue brought

together representatives from Member States, cities, youth, academia, civil society and the UN system, and emphasized the urgent need to place nutrition and health at the core of global food systems transformation. Speakers highlighted the far-reaching implications of our food choices on our health, environment and society, the importance of a common narrative for healthy diets and the great need to invest in nutrition and scale up policy action to yield health, planetary, and economic results.

Controversy Points

Why is this topic controversial?

This topic is heavily linked to the economic state of countries and their people. This is because situations such as political disputes over a long period of time can cause a large number of people to not have access to nutritional food sources; on the other hand, many people who live in peaceful environments do not earn sufficient economic resources to afford nutritionally rich meals three times every day. This problem needs to be resolved, but these kinds of economic problems get in the way and make it almost impossible to fix without changing a large portion of the economic system of the world.

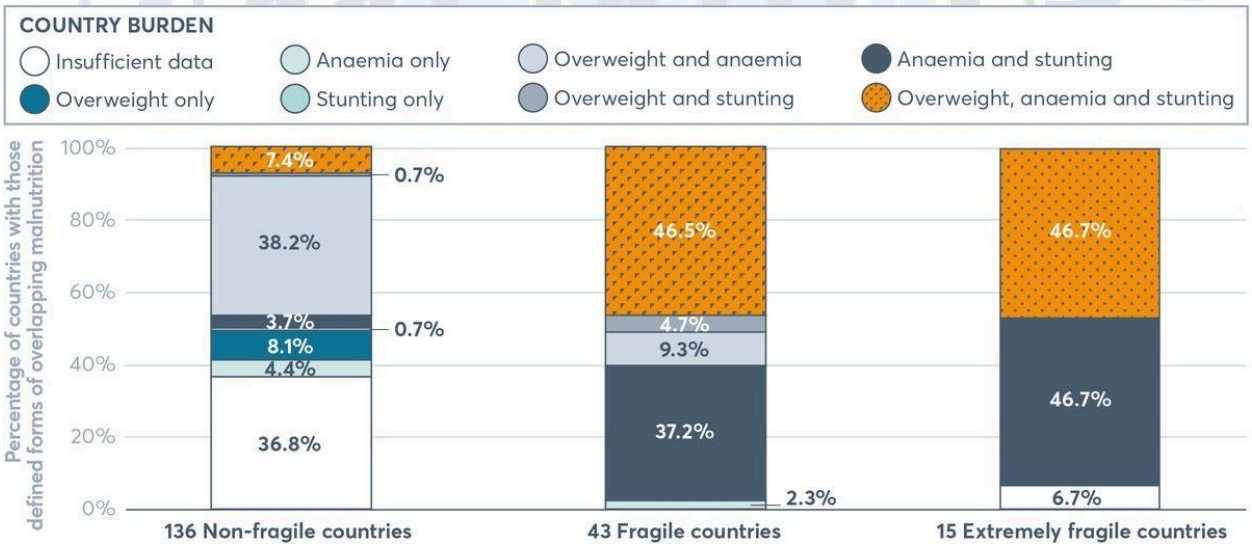
What positions are there on the topic?

Nutrition negligence, encompassing both undernutrition and overnutrition, is a widespread issue with significant global impact, affecting countries across all income levels. Low and middle-income countries face a disproportionate burden of malnutrition, but even wealthier nations grapple with issues like childhood obesity and

micronutrient deficiencies. The consequences of malnutrition are far-reaching, impacting individual health, economic development, and societal well-being.

Some countries actively prioritize nutrition through policies and programs, while others demonstrate negligence through inaction or inadequate measures. This disparity is reflected in the differing positions countries take on the right to food, the implementation of nutrition-related initiatives, and the prioritization of nutrition within their governance structures.

The right to food is a human right, recognized internationally, that ensures everyone has access to adequate food. It is not just about having enough to eat, but also about having access to food that is nutritious, culturally appropriate, and available on a regular basis. Governments tend to have different political stances on the implementation of programs or organizations, as it would require large amounts of funding to create significant changes, which many governments and individuals argue should be invested in more economically productive projects.



In summary, the two main positions are largely dependent on the economic situation of the nation, as developed countries may have a higher advocacy for the right to food, campaigns and organizations, whereas developing countries may not give nutrition the priority it deserves whether due to lack of resources, a lack of political will to implement solutions or other external factors such as needing to urgently attend to other problems the nation may be going through; making it less

of an “against” position, and more of a “secondary problem” situation due to many factors, mainly economical.

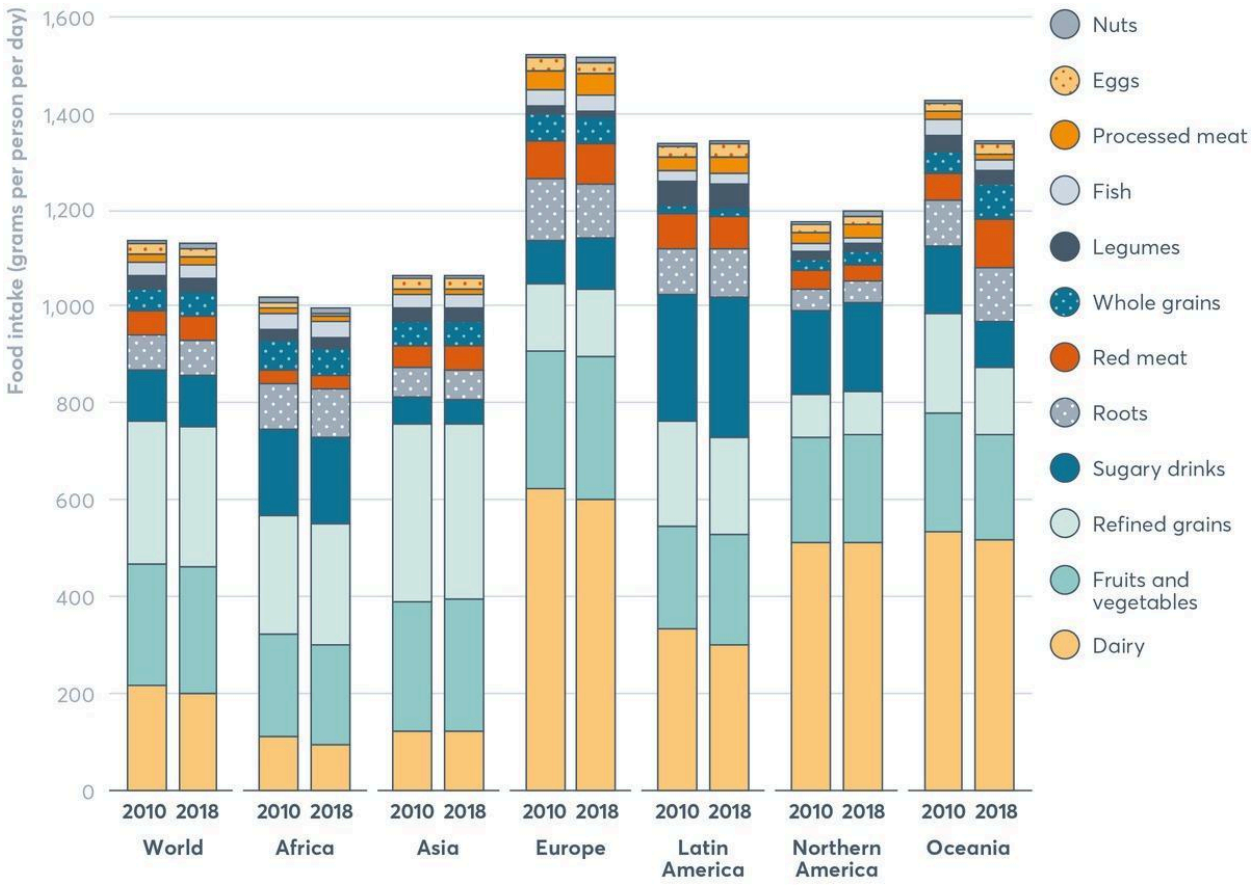
Why is there a debate?

The debate surrounding nutritional negligence stems from the fact that nutrition is an indispensable aspect of a human life, as it affects an individual's whole life. However, even knowing its importance, nutrition is usually overlooked and not taken seriously as a child grows up, resulting in adults who are unable to provide themselves with the necessary nutritional value of every meal. This is primarily caused due to the lack of strong eating habits being created during an individual's childhood.

This neglect of proper nutrition during childhood can start as early as a newborn being breastfed or fed baby formula, as breastfeeding is one of the most effective ways to ensure child health and survival, for it is the ideal food for infants as it is safe, contains antibodies which help protect against many common childhood illnesses; it also provides all the energy and nutrients that the infant needs for the first year of life, as the infant grows up incorporating strong eating habits is crucial to its development and health, emphasizing the need for proper nutrition in the first three years of a child’s life as it helps lower morbidity and mortality, reducing the risk of chronic disease throughout their life span, and promoting regular mental and physical development.

When adulthood begins, children who have not incorporated healthy eating habits and healthy diets into their lives end up as adults who are unable to eat properly, directly affecting their health.

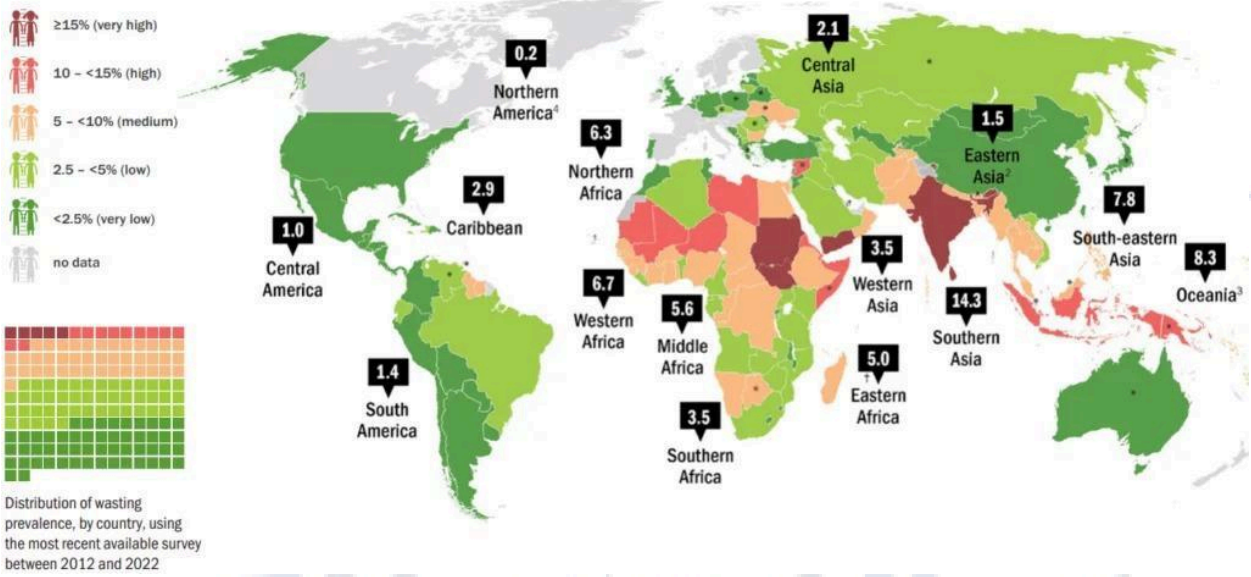
In other extreme situations, food insecurity and overconsumption of food are two other topics that greatly affect the debate surrounding this issue, as all human beings have a right to nutrition and healthcare under the Universal Declaration of Human Rights. Seeking a resolution to all facets of this conflict gives rise to a lively debate that needs to be urgently addressed.


Food intake (grams per person per day)
1,600
1,400
1,200
1,000
800
600
400
200
0
2010 2018
World
Africa
Asia
Europe
Latin America
Northern America
Oceania
Nuts
Eggs
Processed meat
Fish
Legumes
Whole grains
Red meat
Roots
Sugary drinks
Refined grains
Fruits and vegetables
Dairy

Objectives

The World Health Organization and the United Nations seek to reach objectives and resolutions that benefit all. In the specific case of the World Health Organization, striving to reach health for all is the main objective. Taking into account all the resolutions, goals and campaigns, it is important to bear in mind that whilst treating the topic of Nutrition Negligence During Childhood and its Impacts into Adulthood prioritizing diplomatic dialogue that takes into account each country's situation and resources to achieve a common resolution is the key to a productive dialogue that will lead to real change in health for all individuals.

Percentage of children under 5 affected by wasting, by country and United Nations sub-region, 2022



Said dialogue is expected to center on an empathetic approach to tackle the problem at hand, focusing on underprivileged and developing areas where the situation is most intense. According to this, the delegates are expected to provide solutions in order to tackle Nutrition Negligence at its core, such as:

- Eradicate inhumane development situations and conditions, such as a lack of nutritious food, and all cases of malnutrition worldwide.

- Understand and treat the social impact of the problem, its causes, consequences, and how it can affect modern society.

- Create awareness about healthy eating habits during childhood and the impacts if left unattended.

- Create ways to aid people struggling with food disorders and malnutrition in developed countries.

- Adequately implement nutrition programs and food banks, as well as incentivize the public to donate to said causes.

- Address economic issues that cause food insecurity, such as the cost of nutritious food and poor social collaboration, to treat the issue.

- Promote sustainable agriculture and advocate for policies that support strong food supply nets and other organizations that offer aid to people struggling with food insecurity.

- Implement local aid groups that work in collaboration with members of the community to bring meals to people in need as well as collaborate with already in-place shelters and food banks.

- Treat medical conditions caused or worsened by severe malnutrition with specialized care and hospitalization if it warrants healthier conditions.

- Introduce nutritional supplements into the treatment of malnutrition in children, such as vitamins, mineral supplements, and ready-to-use therapeutic food.

Conclusion

The Chair of the World Health Organization has provided its delegates enough context and information in order to develop their own judgment regarding the topic of "Nutrition Negligence During Childhood and its Impacts into Adulthood" that will be seen and discussed during the sessions in the committee. It is important to keep this in mind, as this information will help you, as a delegate, to perform well in the debate.

As seen throughout this document, this topic is connected to a wide array of different problems that add up and affect society in many different ways, such as the economy, health and the productivity of individuals.

Even though this topic is only being presented as an independent problem, it has affected and still affects the way humans have built society. This problem has been noticed for a long time, but it was far too late to improve it without having to heavily affect most of the different sectors of society and possibly destroying them in the process.

As a Chair, we hope the delegates will offer solutions that contribute to eradicating all the unfavorable situations already mentioned, both in this model and in daily life. The Chair wants to emphasize the importance of having an open-minded attitude; a Model of the United Nations must advocate for every human being's health, independently of their race, gender, identity or social condition.

We, as your Chair, would like to thank all the delegates who have chosen to join this committee in this edition of MUNIO and trust in your capacity to lead a fluent debate and reach the resolutions needed to address this problem.

Countries

Republic of India

Malnutrition in India is a serious problem, despite the country's recent decades of economic growth. With one of the highest rates of food inequality in the world, nearly half of Indian children are considered underweight or malnourished by international standards. India also has the highest rate of childhood wasting in the world, at 18.7%. This problem is urgent and significantly impacts the Indian population and its future, limiting the physical, cognitive, and economic potential of future generations.

Nigeria

Nigeria's northern states are the most affected by two forms of malnutrition: stunting and wasting. UNICEF, the UN children's agency, reported in June 2024 that approximately 11 million Nigerian children were suffering from severe child food deficiency. These high rates of malnutrition pose significant challenges to the country's public health and development.

Pakistan

Over the past decade, Pakistan's progress on child malnutrition has not been encouraging. It is a complex, multisectoral problem that presents across a continuum of poor nutrition, from undernutrition due to deficiencies in energy, protein, and micronutrients to child obesity, having both types of malnutrition, under and over. Strategies to tackle both problems need to be enacted urgently, as it is an issue that affects not only the individual's life but also national development.

South Africa

Several factors are fueling the nutrition negligence crisis in South Africa, including poorly constructed food systems that fail to provide children with nutritious meals, families unable to afford food due to the economic state of the world, and a lack of awareness regarding healthy eating habits. A UNICEF report reveals that South Africa is one of 20 countries that account for 65 percent of all children living in severe child

malnutrition globally.

List of Countries:

Permanent members of WHO (G5):

1. French Republic
2. People's Republic of China
3. Russian Federation
4. United Kingdom of Great Britain and Northern Ireland

Countries for WHO 2026

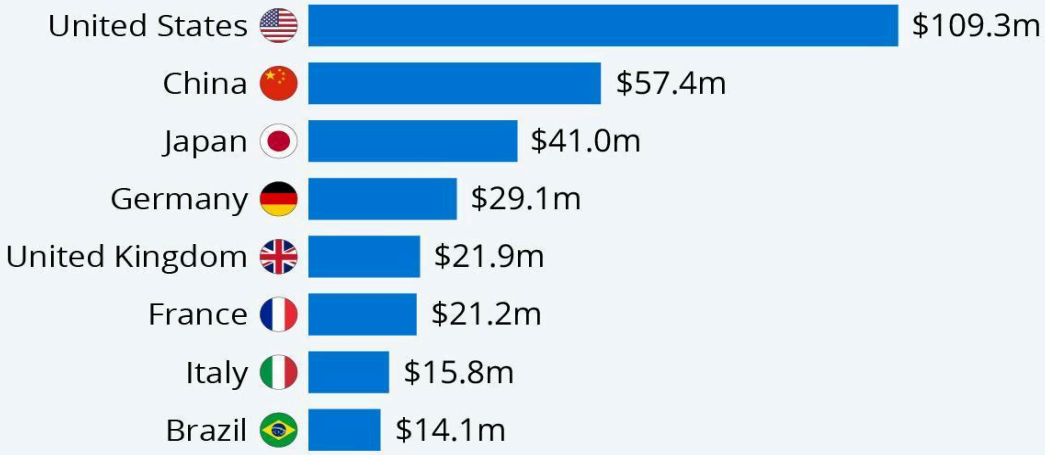
5. Canada
6. Commonwealth of Australia
7. Italian Republic
8. Kingdom of Spain
9. Kingdom of Morocco
10. Kingdom of Saudi Arabia
11. Plurinational State of Bolivia
12. Republic of Colombia
13. Republic of Cuba
14. Republic of Ecuador
15. Republic of Korea (South Korea)
16. Republic of Lebanon
17. Republic of Peru
18. Republic of the Philippines

Countries invited to the debate:

19. Bolivarian Republic of Venezuela
20. Federal Republic of Germany
21. Federative Republic of Brazil
22. Federal Republic of Nigeria
23. Islamic Republic of Afghanistan
24. Islamic Republic of Pakistan
25. Republic of India
26. Republic of South Africa
27. Japan
28. United Mexican States

The Biggest Financial Contributors to the WHO

Assessed contributions to the World Health Organization for 2022*



CHF conversion rate at time of billing
* Calculated relative to a country's wealth and population. Excludes voluntary contributions.
Source: World Health Organization

statista

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"Those who believe in the world are the ones meant to change it".

President: Carlos Sólis Valderrábano

Secretary: José Agustín Carrera Vega

Moderator: Nuria Posada González

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